Vismodegib for advanced basal cell skin cancer

Your diagnosis and treatment setting NCCP 00236: symptomatic metastatic basal cell carcinoma, or locally advanced basal cell carcinoma unsuitable for surgery or radiotherapy.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Vismodegib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Individual anti-sickness / rescue plan	To be completed by the prescribing team if needed	As specified in the individual plan; no routine schedule issued here Source emetogenic category and local policy guide the decision. No medicine, dose or administration order is selected by this draft.

Additional phase-specific support

- Formal pregnancy-prevention programme, nutrition/taste and muscle symptom review. No source-mandated routine creatine phosphokinase (CPK) interval or automatic supplement prescription.
- Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.
- Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-06 · SACTCheck v0.84.40 · Regimen-level drafts; prescriber and oncology-pharmacy verification required